Corporate Credit Memo — CVS Health Corporation (NYSE: CVS)

To: Credit Committee
From: Archit Chawla
Date: October 20, 2026
Re: CVS Health Corporation — senior unsecured credit review; FY2021–FY2025 and twelve months ended June 30, 2026
Companion model: CVS_credit_model.xlsx (live formulas; inputs in data/inputs.json)

Independent research project prepared by Archit Chawla. This memo is not investment advice and is not a credit rating. All financial data are from CVS Health's SEC filings via EDGAR (CIK 0000064803), pulled October 20, 2026.

Recommendation

APPROVE — maintain investment-grade senior unsecured exposure to CVS Health at standard investment-grade terms, subject to the conditions in Section 8. Shadow rating: **BBB, stable outlook.**

The credit has passed its stress point. Adjusted leverage peaked at 4.6x in FY2024, when the Aetna insurance book absorbed a Medicare Advantage cost surge and debt peaked at $66.3B. Since then the company has cut total debt by $4.9B to $61.4B, LTM adjusted EBITDA has recovered to $18.9B, FFO/debt is back to 24.7%, and free cash flow after dividends covered 13.6% of debt in the LTM period. Liquidity is strong and maturities are well laddered. The offsetting facts — a one-notch cushion above high yield at Moody's (Baa3), an earnings base that keeps absorbing litigation charges, and a $5.7B goodwill impairment that confirms overpayment for care delivery — keep this a BBB, not a BBB+.

1. Issuer snapshot

Item	Detail
Issuer	CVS Health Corporation (NYSE: CVS), headquartered in Woonsocket, Rhode Island
Business	Integrated healthcare: health insurance (Aetna), pharmacy benefit management (Caremark), retail/long-term-care pharmacy, primary care delivery (Oak Street Health, Signify Health)
Scale (LTM)	Revenues $415.1B; adjusted EBITDA $18.9B; total assets $253.8B (LTM to Jun 30, 2026)
Capital structure	Total debt $61,410M; cash $11,329M; net debt $50,081M; equity $79,702M (Jun 30, 2026)
Shares out	1,278M (Jun 30, 2026)
Agency ratings	Moody's Baa3, stable; S&P BBB, stable; Fitch BBB, negative — per InvestmentGrade.com ratings database, accessed Oct 20, 2026 (reference only)
Shadow rating	BBB, stable outlook

2. Business profile & competitive position

I view CVS's business profile as the strongest part of the credit. It is one of the largest healthcare companies in the world by revenue ($402.1B in FY2025, +7.8% year over year), with three mutually reinforcing segments (FY2025 segment revenues, before $71.6B of intersegment eliminations):

- **Health Care Benefits (Aetna) — $143.4B.** Health insurance products serving more than 37 million people; premiums were $134.8B of consolidated FY2025 revenue.
- **Health Services — $190.4B.** Caremark PBM (approximately 87 million plan members), specialty pharmacy, and the Oak Street / Signify care-delivery assets.
- **Pharmacy & Consumer Wellness — $139.4B.** Approximately 9,000 retail locations and more than 1,000 walk-in and primary-care clinics; filled 1.8 billion prescriptions (30-day equivalents) in 2025.

Defensive characteristics: demand is non-discretionary and demographically supported; revenue is diversified across insurance premiums, PBM services and pharmacy dispensing; and the vertical model (insurer + PBM + pharmacy + clinics) gives CVS cost and data advantages competitors cannot easily replicate. The structural weaknesses are equally clear: consolidated adjusted EBITDA margin is only ~4–5%, so small swings in medical costs or drug mix move earnings materially; the PBM model faces persistent political and regulatory pressure on rebates and pricing; and insurance earnings are inherently estimate-driven (reserves, premium deficiency assessments).

3. Capital structure

All funded debt is senior unsecured at the parent, plus $1,326M of finance lease liabilities that are presented within the debt lines of the balance sheet (FY2025 10-K, leases note). There is no meaningful secured layer, so unsecured creditors are not structurally subordinated to secured claims at the operating level; the practical subordination is to insurance-subsidiary policyholder obligations, which are ring-fenced by state regulation.

$ millions	Dec 2021	Dec 2022	Dec 2023	Dec 2024	Dec 2025	Jun 2026
Long-term debt, noncurrent	51,971	50,476	58,638	60,527	60,502	59,452
Current portion of long-term debt	4,205	1,778	2,772	3,624	4,068	1,958
Short-term borrowings (commercial paper)	0	0	200	2,119	0	0
Total debt (carrying value)	**56,176**	**52,254**	**61,610**	**66,270**	**64,570**	**61,410**
Cash and cash equivalents	9,408	12,945	8,196	8,586	8,453	11,329
Net debt	**46,768**	**39,309**	**53,414**	**57,684**	**56,117**	**50,081**
Operating lease liabilities	19,823	18,478	17,775	16,650	15,380	14,896

The debt trajectory tells the acquisition story: CVS levered up for Aetna (2018, ~$69B), paid debt down through 2022, re-levered in 2023 to fund Signify Health and Oak Street Health, and peaked at $66.3B at year-end 2024. Deleveraging since then is real, not cosmetic: $3,287M of long-term debt was repaid in H1 2026 alone, commercial paper was taken to zero, and management partly term-funded the stack with $3.0B of junior subordinated hybrid notes issued in December 2024 ($2.25B 7.0% due March 2055; $750M 6.75% due December 2054), instruments structured to receive partial equity credit from the rating agencies. The dividend was held roughly flat ($3,397M paid in FY2025), so deleveraging has been funded from operating cash flow rather than shareholder-return cuts. Revolver covenants are customary and — importantly — contain no acceleration of maturities on a ratings downgrade (FY2025 10-K).

4. Ratio analysis

EBITDA is shown two ways. *Reported EBITDA* is GAAP operating income plus depreciation & amortization. *Adjusted EBITDA* adds back only items that are separate line items in the consolidated statements of operations or discrete charges quantified in the filings' segment reconciliations (goodwill impairment; opioid and legacy litigation charges; loss on assets held for sale; restructuring). I deliberately do **not** add back insurance premium deficiency reserves or medical cost pressure — those are real underwriting economics. FFO is an approximation, stated as such: net income + depreciation & amortization + goodwill impairment.

$ millions, except multiples	FY2021	FY2022	FY2023	FY2024	FY2025	LTM
Revenues	292,111	322,467	357,776	372,809	402,067	415,086
Reported EBITDA	17,796	12,178	18,109	13,113	9,266	12,810
Adjusted EBITDA	18,227	20,514	18,965	14,392	16,531	18,855
Gross leverage (adj. EBITDA)	**3.08x**	**2.55x**	**3.25x**	**4.60x**	**3.91x**	**3.26x**
Gross leverage (reported EBITDA)	3.16x	4.29x	3.40x	5.05x	6.97x	4.79x
Net leverage (adj. EBITDA)	2.57x	1.92x	2.82x	4.01x	3.39x	2.66x
Interest coverage (adj. EBITDA)	7.28x	8.97x	7.14x	4.87x	5.30x	6.08x
FFO (approx.)	12,918	8,535	12,710	9,211	12,099	15,137
FFO / debt	23.0%	16.3%	20.6%	13.9%	18.7%	24.7%
FCF after dividends	13,120	10,543	7,263	2,953	4,410	8,342
Lease-adjusted leverage (adj.)	4.24x	3.52x	4.26x	5.86x	4.92x	4.12x

FY2024 was the trough, and it was an underwriting event, not a demand event. Health care costs rose from $86.2B (FY2023) to $115.1B (FY2024) as Medicare Advantage utilization spiked; Aetna recorded premium deficiency reserves, and segment adjusted operating income in Health Care Benefits collapsed to $307M. Adjusted EBITDA fell to $14.4B against peak debt — 4.60x gross, 5.86x lease-adjusted, FFO/debt of 13.9%. Those are weak-BBB metrics, and Moody's move to Baa3 reflects exactly this period.

FY2025's reported numbers look worse than the credit. GAAP operating income of $4,660M carries the $5,725M Q3 2025 goodwill impairment on the Health Care Delivery reporting unit (non-cash), plus $1,220M of legacy litigation charges and a $320M opioid accrual increase. On the adjusted basis leverage was 3.91x; on reported EBITDA it screens at 6.97x. A committee should hold both numbers in view: the impairment is non-cash, but it is also an admission that acquisition value was destroyed.

The LTM period shows genuine repair. Health Care Benefits adjusted operating income recovered to $2,939M in FY2025 (company segment measure), H1 2026 operating income rose to $9,383M from $5,755M, operating cash flow LTM is $14,780M, and FCF after dividends of $8,342M funded the paydown to $61.4B of debt. LTM FFO/debt of 24.7% and coverage of 6.08x are solidly mid-BBB metrics.

5. Liquidity & maturity profile

Liquidity is a strength. At June 30, 2026 CVS held $11,329M of cash (about $2,700M at the parent or non-restricted subsidiaries; the balance sits largely in regulated insurance entities) plus three undrawn $2.5B back-up revolving credit facilities ($7.5B total, expiring May 2029, 2030 and 2031 per the Q2 2026 10-Q) that backstop the commercial paper program, under which nothing was outstanding. Cash plus revolver capacity of ~$18.8B covers the remaining debt maturities many times over.

Maturity ladder — principal at December 31, 2025 (FY2025 10-K, borrowings note), $ millions:

2026	2027	2028	2029	2030	Thereafter	Total
4,007	3,379	5,008	3,758	4,258	43,299	63,709

68% of principal falls due after 2030 — a long, well-spread ladder with no single-year wall above ~$5.0B (about 8% of principal). The current portion of long-term debt had already fallen to $1,958M by June 30, 2026 after H1 repayments. Scheduled interest on the long-term debt is about $2,933M for 2026 (contractual obligations table). The main scheduled non-debt outflow is the opioid settlement: $3,975M of obligations at December 31, 2025, of which $648M falls within one year — pre-funded by accruals and easily absorbed by LTM operating cash flow of $14,780M.

6. Shadow rating assessment

Shadow rating: BBB, stable outlook (senior unsecured).

- **Business profile (supports BBB+/A-):** dominant, diversified, non-discretionary healthcare platform; $415B LTM revenue; leading positions in PBM, retail pharmacy and a top-tier health insurer.
- **Financial profile (BBB):** adjusted gross leverage of 3.26x LTM (3.91x FY2025) and lease-adjusted leverage of 4.12x sit in the BBB band; FFO/debt of 24.7% LTM and adjusted coverage of 6.08x are mid-BBB; FY2024 showed how fast these metrics can deteriorate (4.60x / 13.9%) when insurance underwriting turns.
- **Governance & policy (neutral to negative):** a decade of debt-funded acquisitions, recurring "one-off" litigation charges (FY2022 opioid $5,803M; FY2025 legacy $1,220M), and the Health Care Delivery impairment argue against a higher assessment despite demonstrated deleveraging since 2024.

My BBB sits in line with S&P (BBB) and Fitch (BBB) and one notch above Moody's Baa3, which I read as pricing the lease-adjusted and trough-period metrics more heavily. **Upgrade path:** adjusted leverage sustained below 3.0x with FFO/debt above 30%. **Downgrade triggers:** adjusted leverage above 4.0x for two consecutive half-years, FFO/debt below 15%, a debt-funded acquisition above $5B, or an adverse litigation judgment materially above accruals.

7. Key risks & mitigants

- **Insurance underwriting volatility.** Medicare Advantage cost trends drove the FY2024 trough; FY2025 still absorbed premium deficiency reserves ($448M in Q1 2025 on the individual exchange book, with further reserves in Q2). *Mitigant:* repricing and benefit redesign flowed through — Health Care Benefits adjusted operating income recovered from $307M (FY2024) to $2,939M (FY2025).
- **Litigation overhang.** Beyond the accrued opioid obligations ($3,975M), CVS carries False Claims Act exposure from the April 2025 Omnicare jury verdict (the $1,220M legacy charges relate to two court decisions on past practices) and, per Dow Jones / Benzinga reports on October 14, 2026, agreed to pay $315M to resolve federal allegations over opioid dispensing. *Mitigant:* charges are accrued as they crystallize and are small relative to ~$15B of LTM FFO; the risk is a judgment materially above accruals, which is a stated re-review trigger.
- **Care-delivery execution.** The $5,725M Health Care Delivery goodwill impairment, $288M of Accountable Care (ACO REACH) losses in FY2025 and clinic closures say the Oak Street / Signify thesis is unproven. *Mitigant:* the impairment is non-cash and the write-down reduces future earnings drag from this unit; group cash flow does not depend on it.
- **PBM and drug-pricing regulation.** Rebate reform, pricing transparency rules and GLP-1 cost inflation pressure Health Services and pharmacy margins; large PBM clients periodically re-bid. *Mitigant:* scale (87M plan members), specialty pharmacy growth, and the integrated model give CVS more offsets than a standalone PBM.
- **Refinancing cost.** Interest expense has risen from $2,287M (FY2022) to $3,119M (FY2025) as cheap pandemic-era coupons roll off. *Mitigant:* the ladder is long (68% of principal after 2030) and LTM coverage is 6.08x on the adjusted basis; no downgrade-acceleration clauses exist in the revolvers or notes.

8. Recommendation & conditions

Approve senior unsecured exposure to CVS Health Corporation at investment-grade terms, sized to a BBB internal assessment, with the following conditions:

- **Exposure cap / re-review trigger:** total debt above $70B, or any single debt-funded acquisition above $5B, triggers a fresh committee review before further purchases.
- **Monitoring tests (semi-annual, on filings):** adjusted gross leverage at or below 4.25x and FFO/debt at or above 15%, computed on the definitions in Section 4. Two consecutive breaches move the internal assessment to BBB- and freeze additions.
- **Litigation watch:** any single judgment or settlement commitment above $1.5B beyond existing accruals triggers re-review.

- **Tenor preference:** favor maturities inside the light 2027–2030 window of the ladder while the deleveraging program completes.

9. Methodology & verification

- **Sources.** SEC EDGAR XBRL company facts for CIK 0000064803, pulled October 20, 2026 (pull_data.py reproduces the pull; raw JSON in data/). Annual figures are the latest-filed 10-K values for each fiscal year; interim figures are from the Q2 2026 and Q2 2025 10-Qs. LTM = FY2025 - H1 2025 + H1 2026; balance-sheet items at period end.
- **Debt definition.** Long-term debt (noncurrent) + current portion + short-term borrowings, at carrying value. This ties exactly to the FY2025 10-K: total borrowings of $64,570M at December 31, 2025 per the borrowings note, and the maturity ladder sums to the $63,709M gross principal disclosed in the same note.
- **Adjustments.** Each add-back is a separate line item in the consolidated statements of operations or a discrete charge from the filings' segment reconciliations: FY2021 goodwill impairment $431M; FY2022 opioid litigation charges $5,803M and loss on assets held for sale $2,533M; FY2023 restructuring $507M and loss on assets held for sale $349M; FY2024 restructuring $1,179M and opioid accrual charge $100M; FY2025 goodwill impairment $5,725M, legacy litigation $1,220M (recorded H1 2025) and opioid accrual charge $320M (Q3 2025). Only the Q3 2025 items fall inside the LTM window.
- **FFO.** Approximation, labelled as such: net income + depreciation & amortization + goodwill impairment. It excludes deferred taxes, stock compensation and other non-cash items that a full agency FFO build would include.
- **Verification.** The companion workbook computes every ratio from the Inputs sheet with live formulas; after building, it was recalculated in LibreOffice and all 132 ratio cells matched an independent Python recomputation from data/inputs.json exactly (within rounding). Input values were additionally tied by hand to the FY2025 10-K balance sheet and borrowings note and to the Q2 2026 10-Q balance sheet (total debt $61,410M; cash $11,329M).
- **Ratings reference.** Agency ratings were verified during preparation from a public ratings database (InvestmentGrade.com, accessed October 20, 2026) and are cited for reference only; the shadow rating is my own assessment.

10. Limitations

- Leverage is measured at period-end balances; intra-period peaks (e.g., commercial paper swings) are not captured. Commercial paper was $2,119M at December 31, 2024 and nil at both December 31, 2025 and June 30, 2026.
- Adjusted EBITDA depends on my judgment about which charges are non-recurring; reported-basis ratios are shown alongside so the committee can apply its own haircuts. Charges described as one-off have recurred in some form in three of the last five years.
- Insurance reserve adequacy (including premium deficiency reserves and the opioid accrual) is taken as reported; I have not independently audited reserves.
- H1 2026 figures are unaudited. June 2026 finance lease liabilities are carried at December 2025 values because the Q2 2026 condensed balance sheet does not break them out ($1,326M; immaterial to the conclusions).
- Figures are as most recently reported in comparative filings; immaterial reclassifications versus originally filed amounts (e.g., FY2022 operating income) are not adjusted.
- Agency ratings are cited from a secondary public database rather than agency releases; outlooks in particular may lag agency actions.
- This memo uses public information only. It is an independent educational project, not investment advice and not a credit rating.